

 Announcement

Kindly note: We're currently experiencing a high volume of inquiries and calls, potentially leading to delays in our initial response.



Dermatology 23S Module 2

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Day 2 Follow on Questions

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by [Derek Kayanja \(Tutor\)](#) - Monday, 20 November 2023, 3:33 PM

Please answer at least one of the questions below (and remember to comment on each others posts and ask questions).

1. Discuss the aetiology of basal cell carcinomas (BCCs).
2. Discuss surgical management of BCCs including Mohs.
3. Discuss histological variants of BCC.
4. Discuss prognosis of BCC.

44 words

[Reply](#)**Re: Day 2 Follow on Questions**

by [Xiao Liew Ding](#) - Tuesday, 21 November 2023, 8:06 AM

Question 1: Aetiology of basal cell carcinomas (BCCs)

Basal cell carcinoma (BCC) is the most common type of skin cancer, and its etiology is primarily associated with cumulative exposure to ultraviolet (UV) radiation, particularly from sunlight (Ludman, P., 2023). Here's a more detailed discussion of the etiological factors of basal cell carcinoma:

1. Ultraviolet (UV) Radiation Exposure:

Sunlight Exposure: The major risk factor for the development of basal cell carcinoma is prolonged and cumulative exposure to ultraviolet radiation from the sun. UV radiation induces DNA damage in the skin cells, leading to mutations that can contribute to the initiation and progression of cancer.

2. Genetic Factors:

Family History: There is evidence to suggest that a family history of skin cancer, including basal cell carcinoma, may increase an individual's susceptibility to developing BCC. Certain genetic syndromes, such as Gorlin-Goltz syndrome, are associated with an increased risk of developing multiple BCCs.

3. Skin Type and Pigmentation:

Fair Skin: Individuals with fair skin, light hair, and blue or green eyes are more susceptible to the harmful effects of UV radiation. This is because melanin, the pigment responsible for skin colour, provides some protection against UV radiation, and people with fair skin have less melanin.

4. Immunosuppression:

Weakened Immune System: Conditions or medications that suppress the immune system may increase the risk of basal cell carcinoma. The immune system plays a crucial role in recognizing and eliminating cancerous cells, and a compromised immune response may allow abnormal cell growth.

5. Environmental Factors:

Exposure to Carcinogens: Other environmental factors, such as exposure to certain chemicals, may contribute to the development of skin cancers. Exposure to elevated levels of arsenic heightens the likelihood of skin cancer development. Arsenic, a naturally occurring element, is present in some regions' well water. Additionally, it is utilized in the production of certain pesticides and in various industries. Individuals working with coal tar, paraffin, and specific petroleum products may also face an elevated risk of developing skin cancer (American Cancer society, 2023).

6. Chronic Inflammation:

Chronic Skin Inflammation: Chronic inflammatory skin conditions, such as scars from burns, may increase the risk of developing basal cell carcinoma. Inflammation can create an environment conducive to the development of cancerous lesions.

7. Ionizing Radiation Exposure:

Medical Radiation: Exposure to ionizing radiation, such as that used in certain medical treatments, has been associated with an increased risk of skin cancer, including basal cell carcinoma.

References:

Ludmann, P. (2023). Skin Cancer Types: Basal Cell Carcinoma Causes. *American Academy Dermatology Association [online]*. Available at: <https://www.aad.org/public/diseases/skin-cancer/types/common/bcc/causes#:~:text=The%20cause%20of%20basal%20cell,as%20tanning%20beds%20or> (Accessed 21/11/2023)

American Cancer Society (2023). *Basal and Squamous Cell Skin Cancer Risk Factors* [online]. Available at: <https://www.cancer.org/cancer/types/basal-and-squamous-cell-skin-cancer/causes-risks-prevention/risk-factors.html#:~:text=Exposure%20to%20certain%20chemicals,-Being%20exposed%20to&text=Arsenic%20is%20an%20element%20found,increased%20risk%20of%20skin%20> (Accessed 21/11/2023)

Discussion:

UV radiation has been the most discussed culprit for skin cancer. What is UV index?

453 words

Reply

**Re: Day 2 Follow on Questions**by [Rahima Abdulaziz Ahmed](#) - Tuesday, 21 November 2023, 11:16 AM

UV INDEX (UVI) ; measures the level of ultraviolet radiation expected on a given day i.e. measure of the strength of the sun's harmful rays at a particular place and time. This index indicates UV radiation which is in turn dictated by many factors such as time of the day, location, latitude, altitude, ozone layer and earth surface characteristics. The UVI also fluctuates throughout the day, however the highest daily value is what gets reported. The reported UVI predicts the UV level at solar noon, which typically occurs between 12pm and 2pm. The higher the number the greater the chances of skin sun damage. (AOSD)

Reference;

American Osteopathic College of Dermatology (online) Ultraviolet index. Available at [https://www.aocd.org/page/UltravioletIndex#:~:text=UVI%20measures%20the%20level%20of,lifetime%20\(Accessed 21/11/23\)](https://www.aocd.org/page/UltravioletIndex#:~:text=UVI%20measures%20the%20level%20of,lifetime%20(Accessed%2021/11/23))

118 words

Reply

**Re: Day 2 Follow on Questions**by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:13 PM

Thank you Rahima, good post.

5 words

Reply

**Re: Day 2 Follow on Questions**by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:13 PM

Thank you Xiao Liey, good and informative post.

8 words

Reply

**Re: Day 2 Follow on Questions**by [Bhavika Patel](#) - Saturday, 23 December 2023, 3:30 PM

Hello Xiao, I just wanted to make some extra points I found to add to your post.

Basal Cell Carcinomas are the common skin cancer seen in Type 1-2 skin types. Aetiology of BCCs are multifactorial and there are a number of risk factors which increase the incidence which are laid out below (Roewert-Huber, J. 2007)

1. **Skin type:** Type 1 skin type have a 30% lifetime risk of developing BCCs.
2. **Sun exposure causing sun damage:** higher incidence in lighter skin tones (Tilli, C.M.L.J. 2005)
3. **Gender:** twice as common in males than females, however higher incidence in women <40 years compared to males < 40 years. Possibly due to high rates of tanning in young females.
4. **UV radiation:**
 - UVB radiation causes DNA/RNA damage through genetic mutations
 - UVA radiation causes photo-oxidative stress when reactive oxygen species are formed after direct exposure to UVA, causing DNA damage.

- PUVA exposure increases risk of developing SCCs
- 5. **Geography:** higher incidence in populations living near the equator. High rates in Australia, US, Europe due to depleting ozone layers allowing more exposure to carcinogenic UV radiation.
- 6. **Toxins:** ingesting arsenic acid, iodine radiation exposure, topical nitrogen mustard application
- 7. **Previous vaccination scarring or thermal burns** can develop secondary BCCs at the same sites
- 8. **Immunosuppression:** increases risk of developing all cancers especially post-transplant patients

In answer to your question on what is UV index. Essentially it is a measure of the strength of sun-exposed UV radiation according to the time and place of exposure. It was developed by epidemiologists to determine the risks of geographical sun exposure causing skin damage and skin cancers. It measures UV wavelengths ranging from 100 to 400 nm and the biochemical responses to skin cells depends on the wavelength and duration of exposure (Feister, U. 2011, Moshhammer, H. 2016).

Low risk: 0-2: little sun protection required

Moderate risk: 3-5: sun protection at peak hours (midday)

High risk: 6-7: sun protection all day

Very high risk: 8-10: sun protection and extra precautions all day, avoid peak hours (midday)

Extreme risk: 11+: avoid sun exposure all day

References:

Roewert-Huber, J., Lange-Asschenfeldt, B., Stockfleth, E. and Kerl, H., 2007. Epidemiology and aetiology of basal cell carcinoma. *British Journal of Dermatology*, 157(s2), pp.47-51.

Tilli, C.M.L.J., Van Steensel, M.A.M., Krekels, G.A.M., Neumann, H.A.M. and Ramaekers, F.C.S., 2005. Molecular aetiology and pathogenesis of basal cell carcinoma. *British Journal of Dermatology*, 152(6), pp.1108-1124.

Moshhammer, H., Simic, S. and Haluza, D., 2016. UV "Indices"—What do they indicate?. *International Journal of Environmental Research and Public Health*, 13(10), p.1041.

Feister, U., Laschewski, G. and Grewe, R.D., 2011. UV index forecasts and measurements of health-effective radiation. *Journal of Photochemistry and Photobiology B: Biology*, 102(1), pp.55-68.

450 words

Reply



Re: Day 2 Follow on Questions

by [Zarmeena Khan](#) - Tuesday, 21 November 2023, 8:30 AM

ETIOLOGY OF BCC:

Basal cell carcinoma (BCC) is the most common type of skin cancer and typically arises in sun-exposed areas of the skin, such as the face, neck, and ears. The aetiology of basal cell carcinoma is multifactorial, with a combination of genetic, environmental, and lifestyle factors playing a role in its development. Here is an overview of some key aspects of the etiology of BCC

1. Ultraviolet (UV) Radiation Exposure:

- The primary environmental risk factor for BCC is exposure to ultraviolet (UV) radiation from the sun or artificial sources like tanning beds. UV radiation induces DNA damage in skin cells, leading to mutations that can contribute to the development of skin cancers, including BCC. Chronic exposure to UV radiation is a major risk factor, and individuals with a history of sunburns or prolonged sun exposure are at an increased risk. (Gallagher RP, 2006)

2. Genetic Factors:

- Genetic predisposition plays a role in the development of BCC. Individuals with a family history of BCC are at a higher risk of developing the disease. Specific genetic syndromes, such as basal cell nevus syndrome (Gorlin syndrome), are associated with a significantly increased risk of multiple BCCs. (Epstein EH,2008)

3. Immunosuppression:

- Individuals with weakened immune systems, whether due to medical conditions (e.g., HIV/AIDS) or immunosuppressive medications (e.g., after organ transplantation), have an elevated risk of developing BCC. The immune system plays a crucial role in detecting and eliminating cancerous cells, and its impairment can contribute to the growth of tumours.

(Lindelöf B, Krynitz B, Ayala F,1996)

4. Other Risk Factors:

- Fair skin, light-colored eyes, and a tendency to burn rather than tan are associated with an increased risk of BCC. Additionally, exposure to certain environmental toxins and ionizing radiation may contribute to the development of skin cancers.

(Karagas MR,1992)

Reference:

- Karagas MR, Stukel TA, Greenberg ER, Baron JA, Mott LA, Stern RS. Risk of subsequent basal cell carcinoma and squamous cell carcinoma of the skin among patients with prior skin cancer. Skin Cancer Prevention Study Group. JAMA. 1992 Oct 14;268(14):1933-8. PMID: 1522844.

-Gallagher RP, Lee TK. Adverse effects of ultraviolet radiation: a brief review. *Prog Biophys Mol Biol.* 2006 Jan;92(1):119-31. doi: 10.1016/j.pbiomolbio.2006.02.011. PMID: 16782325.

- Epstein EH. Basal cell carcinomas: attack of the hedgehog. *Nat Rev Cancer.* 2008 Oct;8(10):743-54. doi: 10.1038/nrc2503. PMID: 18794972.

- Lindelöf B, Krynitz B, Ayala F. Basal cell carcinoma and other cutaneous malignancies in renal transplant recipients. *Transplant Proc.* 1996 Dec;28(6):3371-3. PMID: 8974070

410 words

Reply

**Re: Day 2 Follow on Questions**

by [Oyeombiliyetu Jason](#) - Tuesday, 21 November 2023, 11:19 PM

I would like to further add, though the exact cause of basal cell carcinoma is not known it is associated with, additionally, syndromes such as Xeroderma pigmentosum, nevoid basal cell carcinoma, bazex syndrome, and Rombo syndrome.

It is also associated with alcohol use which may be explained by increased sun exposure in persons under the influence.

hydrochlorothiazide use, due to its property as a potent photosensitiser.

Reference

1. R. Bader(2022). 'Basal cell carcinoma'. Medscape. Available at:

[https://emedicine.medscape.com/article/276624-guidelines#g3?](https://emedicine.medscape.com/article/276624-guidelines#g3?form=fpf&scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf)

[form=fpf&scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf](https://emedicine.medscape.com/article/276624-guidelines#g3?form=fpf&scode=msp&st=fpf&socialSite=apple&icd=login_success_ap_match_fpf). Accessed: 20 November 2023.

81 words

Reply

**Re: Day 2 Follow on Questions**

by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:14 PM

Oyeombiliyetu, thank you for your contribution.

6 words

Reply

**Re: Day 2 Follow on Questions**by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:14 PM

Thank you Zarmeena, good post.

5 words

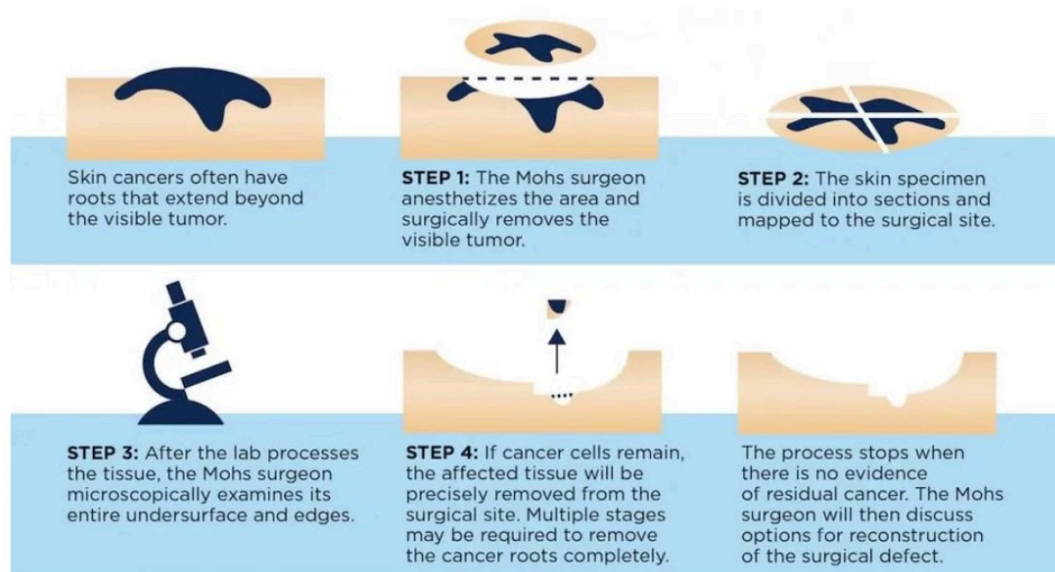
Reply

**Re: Day 2 Follow on Questions**by [Zarmeena Khan](#) - Tuesday, 21 November 2023, 8:54 AM

The **surgical management of basal cell carcinomas (BCCs)** involves various techniques, and the choice of approach depends on factors such as tumour size, location, histological subtype, and the patient's overall health. One widely used and highly effective surgical technique for treating BCCs is Mohs micrographic surgery. Below, I'll discuss the surgical management of BCCs, with a focus on Mohs surgery, supported by relevant references.

1. Mohs Micrographic Surgery (MMS):

Mohs surgery is a specialized technique that allows for the precise removal of skin cancer while preserving healthy tissue. It involves the systematic removal and examination of thin layers of tissue, one layer at a time, until no cancer cells are detected microscopically. This method offers high cure rates and is particularly suitable for tumors with ill-defined borders, aggressive histological subtypes, or tumors located in areas where tissue preservation is crucial.



(Dr. David Roy, 2019)

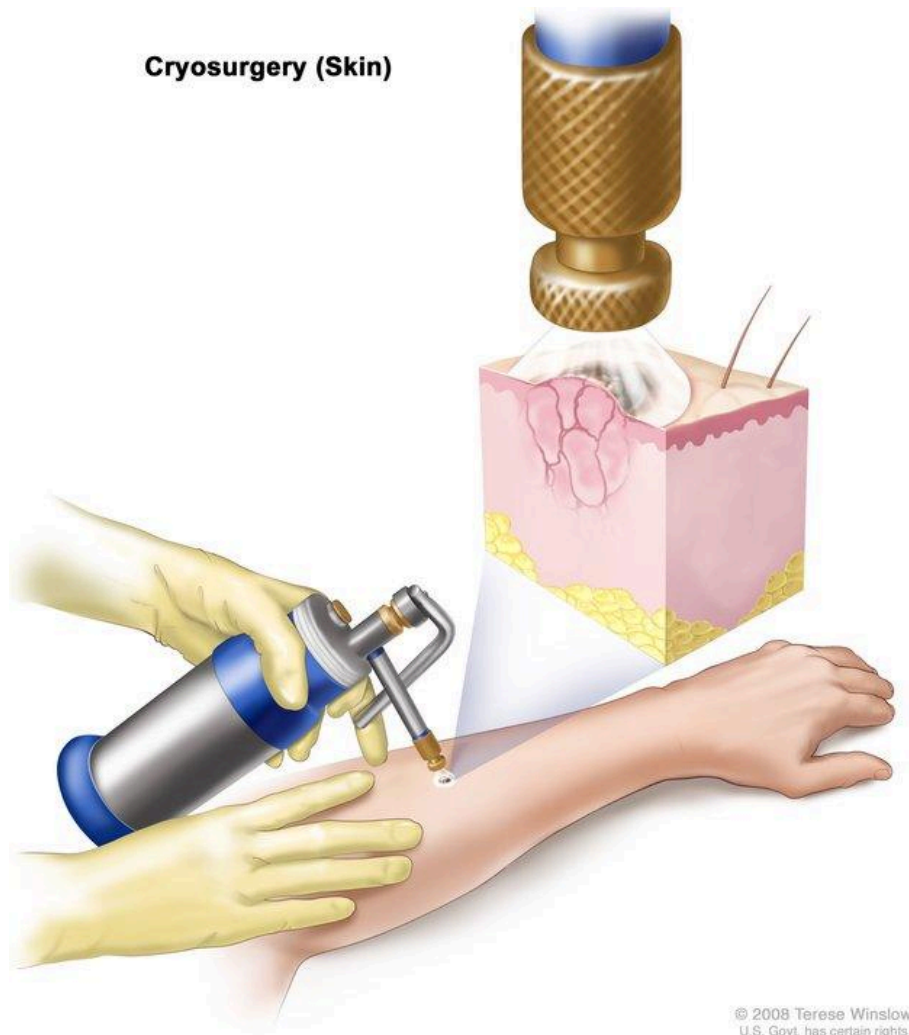
2. Standard Excision:

Standard excision involves removing the tumor with a margin of healthy tissue. This is a common approach for smaller, well-defined BCCs. The excised tissue is then sent for pathological examination to ensure complete removal. (Miller SJ, 1991)

3. Cryosurgery:

Cryosurgery involves freezing the tumor using liquid nitrogen. It is suitable for superficial BCCs and is often used in cases where other surgical options are not feasible.

(Hruza GJ, 1995)



Cryosurgery uses extreme cold produced by liquid nitrogen or argon gas to destroy cancer cells. (National Cancer Institute, 2021)

4. Electrodesiccation and Curettage (ED&C):

ED&C involves scraping off the tumor with a curette and then cauterizing the wound using an electric needle. This method is suitable for small, low-risk BCCs.

(Ratner D, Lowe L, Johnson TM, 2000)

- References:

- Mohs FE. Chemosurgery: *microscopically controlled surgery for skin cancer*. Springfield, IL: Charles C Thomas; 1978.

- Rowe DE, Carroll RJ, Day CL Jr. Mohs surgery is the treatment of choice for recurrent (previously treated) basal cell carcinoma. *J Dermatol Surg Oncol*. 1989 Oct;15(10):1152-8. doi: 10.1111/j.1524-4725.1989.tb03223.x. PMID: 2671681.

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-National Cancer Institute. (2021). *Cryosurgery to Treat Cancer*. [online] Available at: <https://www.cancer.gov/about-cancer/treatment/types/surgery/cryosurgery> [Accessed 21 Nov. 2023].

- Hruza GJ. Techniques and uses of cryosurgery: an update. *J Am Acad Dermatol*. 1995 Feb;32(2 Pt 1):149-63. doi: 10.1016/0190-9622(95)90024-1. PMID: 7822738.

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In summary, the surgical management of basal cell carcinomas encompasses various techniques, with Mohs micrographic surgery being a particularly effective and precise method, especially for tumors with high-risk features. The choice of approach should be individualized based on the characteristics of the tumor and the patient.

545 words

Reply



Re: Day 2 Follow on Questions

by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:15 PM

Thank you Zarmeena, good post and good illustrations

8 words

Reply



Re: Day 2 Follow on Questions

by [Adam Tollitt](#) - Tuesday, 21 November 2023, 9:48 AM

4. Discuss the prognosis of BCCs.

Broadly speaking, the prognosis for BCCs is very favourable, with a large majority of BCCs being fully cured by treatment (Oakley, 2015). Furthermore, BCCs rarely metastasise, with estimated incidences of histologically confirmed metastatic BCCs ranging between 0.0028-0.55% (Queirolo et al., 2023).

Morbidity related to BCCs is linked to its locally invasive and destructive effects and potential risk of recurrence after initial treatment. The risk of recurrence within 3 years of a first BCC is high with estimates in North European populations ranging from 25-44% (Marcil and Stern, 2000; Schreuder et al., 2021).

The risk of BCC recurrence is linked to both is location and clinical/histopathological characteristics.

BCC location and risk of recurrence:

Low risk locations = Trunk, limbs

Intermediate risk locations = Forehead, chin, cheeks, scalp, neck

High risk locations = Centrofacial areas, nose, ears, perioral area, embryonic fusion planes

(Armstrong, Magnusson and Guppy, 2016; McDaniel, Badri and Steele, 2022)

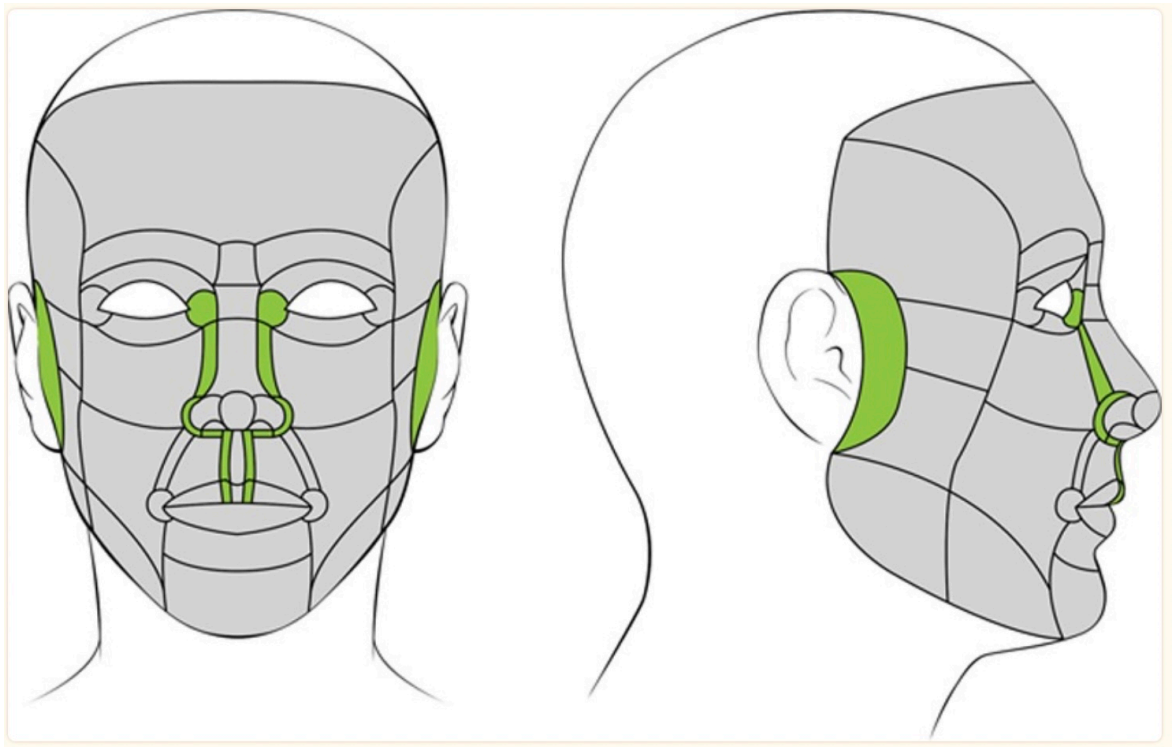


Figure 1 – Anatomical diagram of the face showing the facial units and embryonic fusion planes (highlighted in green – medial canthus, paranasal areas, ala crease, inferior naris, philtral ridge, vertical lip, preauricular areas) (Armstrong, Magnusson and Guppy, 2016).

BCC clinical/histological features and related prognosis:

Good prognosis/low recurrence risk = primary superficial BCCs, primary nodular BCCs <1cm in intermediate-risk area or <2cm in low-risk area

Intermediate prognosis/intermediate recurrence risk = recurrent superficial BCCs, nodular BCCs <1cm in high-risk area OR <2cm in intermediate-risk area OR >2cm low-risk area

Poor prognosis/high recurrence risk = nodular BCCs >1cm in high risk area, recurrent BCC, morpheaform, infiltrative or histologically aggressive

(McDaniel, Badri and Steele, 2022)

References

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doi:<https://doi.org/10.1097/GOX.0000000000000571>.
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doi:<https://doi.org/10.1001/archderm.136.12.1524>.
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and recommendations by the Italian Association of Medical Oncology. *ESMO Open*, 8(6), pp.102037–102037. doi:<https://doi.org/10.1016/j.esmoop.2023.102037>.

Schreuder, K., Hollestein, L., Nijsten, T.E.C., Wakkee, M. and Louwman, M.W.J. (2021). A nationwide study of the incidence and trends of first and multiple basal cell carcinomas in the Netherlands and prediction of future incidence*. *British Journal of Dermatology*, 186(3), pp.476–484. doi:<https://doi.org/10.1111/bjd.20871>.

475 words

Reply



Re: Day 2 Follow on Questions

by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:16 PM

Thank you Adam, very useful addition to our discussion.

9 words

Reply



Re: Day 2 Follow on Questions

by [Adam Tollitt](#) - Tuesday, 21 November 2023, 10:42 AM

Follow on questions for the group following my post about BCC prognosis:

In the UK, the national standard for BCC referral for specialist secondary care/treatment is a routine referral; unless there is clinical concern about a delay in referral causing significant impact due to the lesion site/size, in which case a suspected cancer pathway (2 week wait/review referral) may be considered (NICE, 2021).

I am interested to hear about the referral pathways/urgency of specialist review for BCCs in other countries/healthcare systems; does anyone have any further information about this?

Have any UK based primary care clinicians in the group encountered any challenges explaining to patients that although they have a cancerous skin lesion, a referral for specialist review/care via a cancer referral pathway is not indicated?

References

National Institute for Health and Care Excellence, UK (NICE). (2021). *Referral for suspected skin cancer*. [online]. Available at: <https://cks.nice.org.uk/topics/skin-cancers-recognition-referral/management/referral-for-suspected-skin-cancer/>. (accessed 21/11/2023).

149 words

Reply



Re: Day 2 Follow on Questions

by [Pooran Outar](#) - Tuesday, 21 November 2023, 1:40 PM

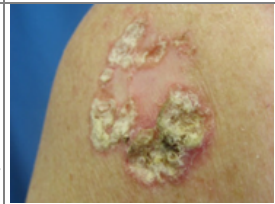
Aetiology:

The precise origin of BCC remains uncertain, but it arises from melanocytes and is primarily linked to prolonged UVB and UVA exposure. This exposure induces DNA mutations in the patched tumor suppressor gene, a component of the hedgehog signaling pathway (Oakley, 2015).

Surgical Management:

Mohs micrographic surgery, is the technique of choice to remove BCC ensuring meticulous microscopic margin examination, higher cure rates, tissue preservation, and superior cosmetic results. Other surgical alternatives, such as standard excision and electrodesiccation with curettage and cautery, are suitable for superficial BCCs (Newlands et al., 2016).

Histological Variants:

Types of BCC	Characteristics	Related Image
Nodular	Most common type of facial BCC Shiny or pearly nodule with a smooth surface May have central depression or ulceration, so its edges appear rolled Blood vessels cross its surface Cystic variant is soft, with jelly-like contents Micronodular, microcystic and infiltrative types are potentially aggressive subtypes	 (Oakley, 2015)
Superficial	Most common type in younger adults Most common type on upper trunk and shoulders Slightly scaly, irregular plaque Thin, translucent rolled border Multiple microerosions	 (Oakley, 2015)
Morphoeic	Usually found in mid-facial sites Waxy, scar-like plaque with indistinct borders Wide and deep subclinical extension May infiltrate cutaneous nerves	 (Oakley, 2015)
Basosquamous	Mixed basal cell carcinoma and squamous cell carcinoma Infiltrative growth pattern Potentially more aggressive than other forms of BCC	 (Oakley, 2015)

Prognosis:

The majority of BCC cases are successfully treated, with the highest likelihood of cure when addressing small lesions. Approximately 50% of individuals with BCC experience the development of a second lesion within three years of the initial occurrence. There is also an elevated risk of other skin cancers, particularly melanoma. In a minority of cases, the cancer may reappear in the skin, necessitating additional treatment (Cancer Research UK, 2023); (Oakley, 2015).

References:

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- Newlands, C., Currie, R., Memon, A., Whitaker, S. and Woolford, T. (2016). Non-melanoma skin cancer: United Kingdom National Multidisciplinary Guidelines. *The Journal of Laryngology & Otology*, [online] 130(S2), pp.S125–S132. doi:<https://doi.org/10.1017/s0022215116000554>.

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352 words

Reply

**Re: Day 2 Follow on Questions**by [Derek Kayanja \(Tutor\)](#) - Wednesday, 22 November 2023, 5:17 PM

Thank you Pooran, good post and nice pictures.

8 words

Reply

**Re: Day 2 Follow on Questions**by [Felica Mwinji Namukonde](#) - Friday, 24 November 2023, 11:41 AM

SURGICAL MANAGEMENT OF BCCs including Mohs:

mostly recommended in first-line therapy (standard surgical excision) for primary nodular or superficial basal cell carcinoma <20mm in diameter on sites like trunk and extremities this excludes genitalia, pretibia hands and feet. preoperative margin evaluation is important in this case 4 to 5mm margins are appropriate. Repair surgical defect by us of side to side closure or use of adjuvant flaps or skin grafts. Factors that determine type of closure include size, depth and location. Efficacy of standard surgical excision compared to non surgical treatment of BCCs on recurrence rate is lower.

Advantage of Standard excision is that it faster than Mohs surgery and not expensive gives information on surgical margins performed on local anaesthesia and well tolerated (Aaasi S Z 2023).

Mohs micrgraphic surgery: usually used in treating BCCs at high risk of recurrence this is also recommended for tumors in high risk areas like face (mask area) this procedure allows 100% control of tumor margins thereby minimizing resection of normal tissue, apart from that it allows in identifying inapparent tumor extensions thereby increasing the sparing of normal tissue.

Staged excision with circumferential margin assessment (CCPDMA) : An alternative to MMs for high risk BCCs done by fully assessing tissue margins by using formalin-fixed tissue rather than the rapidly processed frozen sections in standard MMS

Efficacy of large BCCs was associated with 5 year cure rate.

close follow-up is required to dertermine new skin cancers and recurrences six monthly review is recommended (Sumaira ZA)

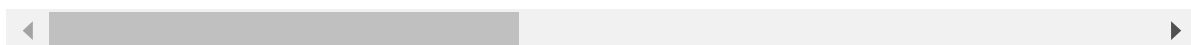
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Kerly JR, Patel A.J.K A (2006) retrospective study of 694 Basal Cell Carcinoma excisions to quantify deep margin documentation and clearance compared to histological type and surgical margin Available at: https://findit.southwales.ac.uk/discovery/fulldisplay?docid=cdi_proquest_miscellaneous_2310714918&context=PC&vid=44WHELF_USW:44WHELF_USW_NUI1&lang= Accessed (24/11/23)

294 words

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**Re: Day 2 Follow on Questions**by [Derek Kayanja \(Tutor\)](#) - Saturday, 25 November 2023, 3:24 PM

Thank you Felica, good contribution.

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